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CT ANGIO HEAD

Collected on Aug 17, 2024 6:15 AM

Is there incorrect information on this page?

Results

Impression

Impression:
Within the limitations of the microvascular ischemic white matter disease and chronic left thalamic and bilateral basal ganglia infarcts, there is no acute intracranial pathology. MRI is more sensitive for evaluation of an acute infarct.

Findings were conveyed to the emergency department using the preliminary report function on Stentor on 8/17/2024 6:40 AM.

CTA of the head & neck:
CTA images of the neck and head were obtained after IV infusion of 100 cc of Isovue 370. Rotating MIPS and reformats were generated and reviewed.
Comparison: None
Clinical history: Facial paresthesia, hypertension
Findings:

There is no significant stenosis along the right subclavian artery. There is marked segmental stenosis along the right vertebral artery V4 segment. There is marked stenosis of the left vertebral artery at its origin. The left vertebral artery is occluded at the V3/V4 junction. However, the left posterior inferior cerebellar artery is adequately opacified. Short basilar proximal segment is opacified. However, the remaining basilar artery opacification is lost. The right and left posterior cerebral arteries are adequately opacified via the dominant posterior communicating arteries.

There is moderate segmental narrowing of the right ICA supraclinoid segment.

There is no hemodynamically significant stenosis along the cervical segments of the right or left carotids. There is no significant narrowing along the left ICA intracranial segments. However, there are moderate focal and segmental narrowings along the left middle cerebral artery M1 segment.

There is no evidence of an intracranial aneurysm or venous sinus thrombosis.

Impression:

* There is marked segmental narrowing along the right vertebral artery V4 segment. The left vertebral artery is occluded at the V3/V4 junction. Only short proximal basilar artery segment is opacified. The remaining basilar artery is not seen. The right and left posterior cerebral arteries are adequately opacified via posterior communicating arteries.
* Moderate right ICA supraclinoid segment secondary to calcified atherosclerotic plaques.
* Moderate focal and segmental narrowings along the right middle cerebral artery M1 segment are seen.

Signed by: Hossam Hamda on 8/17/2024 6:40 AM

Narrative

CT head:
Contiguous axial unenhanced CT sections of the head were obtained from the skull base through the vertex. Images were reviewed in soft tissue and bone algorithms.
Comparison:None
Clinical history: Facial paresthesia, hypertension
Findings:

Chronic left thalamic infarction is noted. Bilateral basal ganglia chronic lacunar infarcts are seen. Periventricular and subcortical presumably microvascular ischemic white matter changes are seen. There is no depressed skull fracture. The scans through the orbits are unremarkable. The scans through the paranasal sinuses, mastoid air cells, and middle ears are significant for wall down right mastoidectomy remote changes.

Authorizing provider: Dr. Shirley Shuppe, DO
Reading physician: Dr. Hossam Hamda, MD
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